Psychological injury treatment, anywhere in WA

Lauren Lynch · Accredited Mental Health Social Worker (AMHSW) · ANZAED Credentialed Eating Disorder Clinician

Fifteen years of clinical practice, with the workers who are hardest to place well: neurodivergent, LGBTQIA+, Aboriginal and Torres Strait Islander, and regional and remote.

WORKER CONTACTED	FIRST APPOINTMENT OFFERED	REPORTS, FROM REQUEST
1 working day	**5** working days	**5** working days

That is the commitment, and it holds in busy weeks too. If a referral would fall outside it, I tell you the day it arrives.

TELEHEALTH

Every worker in the State is in range. A worker in Karratha, Esperance or Derby often waits because there is nobody local. By telehealth, treatment starts on the same timeline wherever they live, billed on the same Schedule 6 codes.

ELIGIBILITY

Ready for your referral today. Accredited by the Australian Association of Social Workers as a mental health social worker, accreditation no. 461419, current to 30 June 2027. That accreditation is the eligibility requirement for mental health social work under regulation 32 of the Workers Compensation and Injury Management Regulations 2024, and Schedule 6 of the Health Services Fees Order pays against it.

COST

The psychology rate, below the clinical and counselling schedules. Schedule 6 sits at the same scheduled hourly rate as a generally registered psychologist, and on the current Order roughly a quarter below the clinical and counselling psychology schedules. Billed at schedule, with no gap to the worker on an accepted claim. Once liability is accepted, accounts go to the insurer or self-insurer against the claim number.

DEFERRED LIABILITY

Treatment can start while liability is decided. Where the insurer gave a deferred decision notice within 14 days and has not decided liability by day 28, provisional payments cover medical and health expenses from the day of injury, up to a cap, until the decision. Treatment given before acceptance is payable once the claim is accepted. I confirm with the insurer where accounts go, so the worker starts straight away.

WHAT I TREAT

Psychological injury arising from work, or compounded by it. Post-traumatic stress. Anxiety and panic. Depression. Adjustment after injury, investigation or a performance process. Secondary psychological injury after a physical injury, including pain-related distress. Sleep. Anger where it threatens the return to work.

SCOPE

Clear from day one. Certificates of capacity stay with the treating medical practitioner, and I refer those requests straight back. Independent medical examinations and permanent impairment assessments sit with other providers, so treating and assessing roles stay separate. If a referral needs something outside my scope, I say so and tell you who does it.

HOW I WORK

Matched to the presentation and the national guidelines. For PTSD, trauma-focused cognitive behavioural therapy, a first-line treatment in the Australian PTSD guidelines, which suggest it by video where in-person care is unavailable. Acceptance and Commitment Therapy and Dialectical Behaviour Therapy skills carry the adjustment, distress tolerance and values work a return to work runs on. I work to the Clinical Framework for the Delivery of Health Services, which WorkCover WA endorses.

MEASURES AND SAFETY

At baseline and at every review. The PCL-5 where trauma is present. DASS-21, K10 or PHQ-9 where they fit. The Work and Social Adjustment Scale (WSAS) for function, because return to work turns on function as well as symptoms. Risk is assessed at intake and every session, with a safety plan where needed and the worker's local crisis and medical supports confirmed before telehealth starts.

WORKING WITH YOU

Reporting is agreed at referral. Before insurer approval, Schedule 6 allows three reports per claim and communication of up to 30 minutes per conversation and 60 minutes across the claim. In line with WorkCover WA's November 2026 guidance, I send a Treatment Management Plan wherever a worker is likely to need more than five consultations. We agree the reporting schedule up front, and I ask before going past it. Reports follow the Schedule 6 specification and are copied to the treating medical practitioner. Treatment goals are set with the doctor and the workplace rehabilitation provider to match the return-to-work plan. I request the next block before the current one runs out, so treatment keeps moving. The practice absorbs missed sessions, and after two in a row you hear from me.

PARTICULAR EXPERIENCE

Eating disorders and body distress, treated with CBT-E, including where an injury or a long claim has disturbed eating. I am a Yorta Yorta and Boonwurrung woman, so cultural safety is something I practise rather than describe.

WHAT TO SEND

Contact and claim details are enough to start. The worker's name and phone number, the claim number, the insurer or self-insurer, the claims manager, and the worker's consent to the referral. A doctor's letter with the claim number also works.

MAKE A REFERRAL

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ABN 76 208 028 254 · AASW accreditation no. 461419 · Professional indemnity and public liability current · NDIS Worker Screening cleared